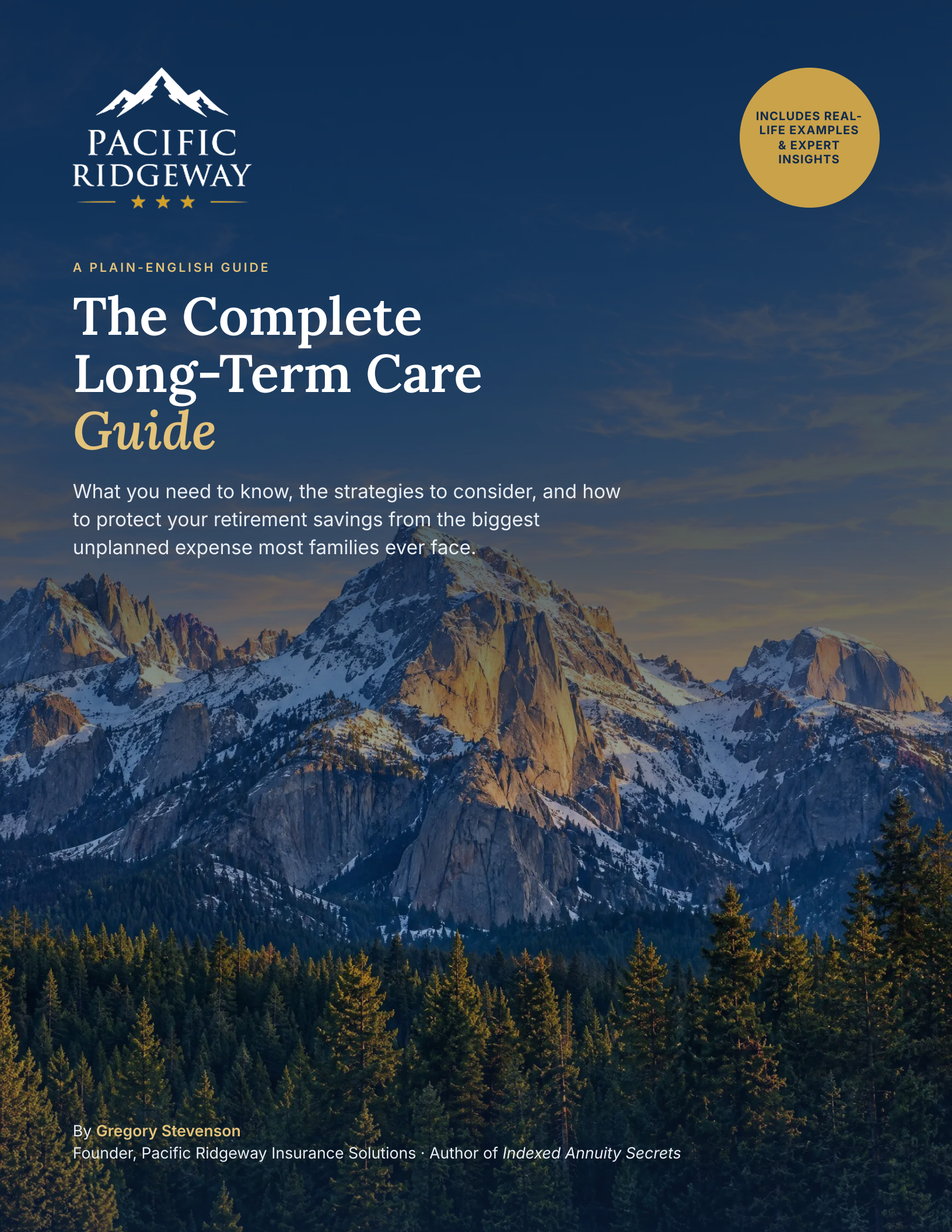
PACIFIC RIDGEWAY

INCLUDES REAL-LIFE EXAMPLES & EXPERT INSIGHTS

A PLAIN-ENGLISH GUIDE

The Complete Long-Term Care *Guide*

What you need to know, the strategies to consider, and how to protect your retirement savings from the biggest unplanned expense most families ever face.



By **Gregory Stevenson**
Founder, Pacific Ridgeway Insurance Solutions · Author of *Indexed Annuity Secrets*

WELCOME

The bill nobody plans for

Seven out of ten people who reach 65 will need some kind of long-term care. Almost none of them have a plan to pay for it. This guide fixes that, in plain English, in about twenty minutes.

Long-term care is not medical care. It is help with everyday living: bathing, dressing, eating, getting around. Medicare does not pay for it. Health insurance does not pay for it. That leaves your savings, your family, or a plan you set up ahead of time.

My mission is simple: help families protect, grow and pass on their wealth with clear answers instead of sales pitches. Our team includes a Certified Long-Term Care consultant, and everything in these pages comes from real planning conversations with real families.

70%
of people turning 65 will need long-term care at some point

$10,600
median monthly cost of a private nursing-home room (2024)

$0
what Medicare pays for ongoing custodial care

Inside this guide

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How to read this guide: every chapter ends with "the simple version." If you only read those boxes, you'll still know more than most people who are already paying for care.

CHAPTER 1

What long-term care really is

It is help with the basics of daily life when you can no longer do them safely on your own. It can last months or many years.

The six "activities of daily living"

Insurance companies and doctors measure need the same way. When someone can't do **two of these six** without help, or has a cognitive impairment like Alzheimer's, they qualify for care under most policies:

BATHING

Getting in and out of a tub or shower safely.

DRESSING

Putting on clothes, braces or prosthetics.

EATING

Feeding yourself once food is prepared.

TRANSFERRING

Moving from bed to chair, standing up.

TOILETING

Getting to and from the bathroom.

CONTINENCE

Managing bladder and bowel function.

Where care happens

Setting	What it is	Who it fits
Home care	An aide comes to your house for a few hours or around the clock	Most people's first choice; 70%+ of care starts at home
Assisted living	Apartment-style community with meals, help and staff on site	Needs help daily but not skilled nursing
Memory care	Secured assisted living for dementia	Alzheimer's and other cognitive decline
Nursing home	24-hour skilled nursing	Complex medical needs or late-stage care

How long does it last?

The average need is about **three years**. Women average longer (3.7 years) than men (2.2 years) because they live longer and usually outlive the spouse who would have cared for them. One in five people will need care for **more than five years**, and that is the group that drains a retirement.

The simple version: long-term care is help with daily living, it usually starts at home, it averages three years, and the "two of six" test is how you qualify for benefits.

CHAPTER 2

What it costs

These are national medians from the 2024 Genworth/CareScout Cost of Care Survey. Big-city and coastal costs run 20–50% higher.

Type of care	Per month	Per year	3-year need
Home health aide (44 hrs/wk)	$6,480	$77,800	$233,000
Homemaker services	$6,290	$75,500	$226,000
Adult day health care	$2,170	$26,000	$78,000
Assisted living	$5,900	$70,800	$212,000
Nursing home, semi-private	$9,280	$111,300	$334,000
Nursing home, private room	$10,650	$127,800	$383,000

Source: Genworth/CareScout Cost of Care Survey 2024, national medians, rounded. Your state and city will differ.

The number that matters: inflation

Care costs have risen roughly **4–5% a year** for two decades, faster than general inflation, because care is labor and labor is scarce. If you are 55 today and need care at 80, multiply today's numbers by about **2.5**.

TODAY, AGE 55

Private nursing room: **$10,650/month**
Three years: about **$383,000**

AGE 80, AT 4% INFLATION

Private nursing room: **about $28,000/month**
Three years: about **$1,000,000**

Why this hits couples hardest

When one spouse needs care, the other still has to live: same mortgage, same groceries, same property taxes. The household goes from one budget to two, and the "well" spouse often ends up with a fraction of what the couple saved. Planning for care is really planning for the spouse who is left.

The simple version: figure $380,000 today for a typical three-year need, closer to $1,000,000 if you're 25 years away from needing it, and remember the healthy spouse still has bills.

CHAPTER 3

Who actually pays

Most families assume "the government" or "my health insurance" covers this. Here is what each really does.

Source	What it pays	The catch
Medicare	Up to 100 days of *skilled* rehab after a 3-day hospital stay	Days 21–100 have a daily copay (~$210 in 2025). Pays **nothing** for ongoing help with daily living.
Medicare Advantage	Some plans add small home-help benefits	Limited hours, limited networks, changes every year
Health insurance	Medical treatment only	Custodial care is excluded
Medicaid	Nursing-home care, some home care	Only after you spend down to roughly **$2,000** in countable assets (varies by state). Limited choice of facilities. 5-year look-back on gifts.
VA benefits	Aid & Attendance pension for wartime veterans and surviving spouses	Income and asset limits; a few thousand dollars a month at most
Your savings	Everything, until it runs out	This is the default plan for most Americans
Your family	Unpaid care, usually from a daughter or daughter-in-law	Lost wages, health strain, family conflict

The Medicaid trap in one paragraph

Medicaid does pay for long-term care, but it is a safety net for people who have little left. To qualify, a single person generally must spend down to about $2,000 in countable assets. A married couple can protect more for the spouse at home, but the limits are strict and the rules are state by state. Anything you gave away in the previous five years can be counted against you. And once you're on Medicaid, you go where there is a Medicaid bed available, not where you'd choose.

> **A common misunderstanding:** "I'll just transfer the house to the kids." Transfers inside the five-year look-back trigger a penalty period during which Medicaid will not pay, and the house may no longer be yours to live in or borrow against. Talk to an elder-law attorney before moving any asset.

The simple version: Medicare won't. Health insurance won't. Medicaid will, but only after you're nearly broke. Everyone else is paying with their own money or their family's time.

CHAPTER 4

Your five ways to pay

There is no single right answer. Most well-planned families use two of these together.

Option	How it works	Best for	Watch out for
1. Self-fund	Set aside investments earmarked for care	$3M+ in liquid assets, comfortable with the risk	Market timing; selling assets in a down year; spouse's lifestyle
2. Traditional LTC insurance	Annual premium buys a daily/monthly benefit pool	Maximum benefit per premium dollar; healthy 50s–early 60s	Premiums can rise; use-it-or-lose-it; underwriting is strict
3. Hybrid life + LTC	A life policy with a long-term care rider; one payment or 10 pays	People who hate "wasting" premium; families who want a death benefit either way	Larger up-front dollars; benefit pools usually smaller than traditional
4. Annuity with LTC rider	A fixed annuity that doubles or triples its payout for qualified care	Health issues that block other coverage; money already sitting in CDs or an old annuity	Lower leverage; benefit tied to account value
5. Life insurance living benefits	Accelerate the death benefit for chronic illness	Anyone already buying life insurance	Reduces what family receives; not a full LTC plan on its own

The leverage question

Every option is really a question of leverage: how many dollars of care does each dollar you commit buy? Self-funding is 1-to-1. A well-designed hybrid policy bought in your 50s often turns one dollar into three to five dollars of care benefit. Traditional coverage can be higher still. That leverage is the entire reason to plan early.

1×
Self-funding: a dollar saved pays a dollar of care

3–5×
Typical hybrid policy bought in your 50s

5–8×
Traditional LTC insurance, healthy applicant, early claim

The simple version: the five tools are savings, traditional insurance, hybrid life/LTC, annuity riders and living benefits. Pick based on your health, your assets and how you feel about premium you may never use.

CHAPTER 5

Traditional long-term care insurance

The original product: you pay a premium each year, and if you need care, the policy pays a monthly benefit until the pool is used up.

The four dials you set

MONTHLY BENEFIT

How much the policy pays per month of care. Match it to the cost of care in *your* area, not the national average.

BENEFIT PERIOD

How long it pays: 2, 3, 5 years or a dollar pool. Three years covers the average; five covers most.

ELIMINATION PERIOD

The waiting period before benefits start, usually 90 days. You cover the first 90 days yourself.

INFLATION PROTECTION

3% compound is the standard. Without it, a benefit bought at 55 covers about a third of the cost at 85.

Pros

- Highest benefit per premium dollar
- Premiums may be tax-deductible (age-based limits; business owners can often deduct more)
- "Partnership" policies in most states let you keep extra assets if you ever apply for Medicaid
- Shared-care riders let couples pool benefits

Cons

- Use it or lose it: no care, no payout, no refund
- Premiums are not guaranteed; older policies saw 50–100% increases
- Strict health underwriting; roughly 1 in 3 applicants over 60 is declined
- Fewer carriers offer it every year

What it costs

For a healthy couple, both 55, buying about $165,000 of benefit each with 3% compound inflation, combined premiums run roughly **$3,000–$4,500 a year** (American Association for Long-Term Care Insurance 2024 price index). The same coverage at 65 costs about 60–80% more, if you can still qualify.

The simple version: traditional coverage is the most efficient if you're healthy and buy in your 50s, but you must be comfortable that premiums can rise and that you may never collect.

CHAPTER 6

Hybrid life + long-term care

The fastest-growing way to plan, and the one most of our clients choose. It answers the biggest objection to traditional coverage: "What if I never use it?"

How it works

A hybrid policy is permanent life insurance with a long-term care benefit built in. You fund it once, or over 5, 10 or 20 years. Then one of three things happens:

YOU NEED CARE

Pays a monthly benefit, typically 2–3% of the death benefit, often for 4–7 years. Many add a rider that keeps paying after the death benefit is used up.

YOU NEVER NEED CARE

Your family receives the full, income-tax-free death benefit. Nothing was "wasted."

YOU CHANGE YOUR MIND

Most contracts return most or all of your premium if you cancel. Your money was parked, not spent.

Why it has become the default

- **Guaranteed premiums.** The price cannot go up. Ever.
- **Easier health approval** than traditional coverage, especially for single-premium designs.
- **Three exits, all of them good:** care benefit, death benefit, or your money back.
- **Tax-free benefits.** Qualified long-term care payments are received income-tax-free under IRC §7702B.

Three ways families fund it

Funding source	How	Why it works
Cash or CDs	Move "safe money" that's earning 1–4%	Turns idle money into 3–5× leverage with a return-of-premium floor
Old life insurance	1035 exchange from an old whole life or universal life policy	No tax on the gain; a policy you bought for the kids 25 years ago becomes your care plan
IRA / 401(k)	Roll a portion into an IRA-funded hybrid, paid over 10 years	Required distributions you'd pay tax on anyway now buy tax-free care benefits

The simple version: hybrid coverage is life insurance that pays for care if you need it and pays your family if you don't, with a premium that can't go up. It's why "use it or lose it" is no longer a reason to skip planning.

CHAPTER 7

Strategies high-net-worth families use

Wealthy families rarely buy care insurance because they *can't* pay. They buy it because it is cheaper than paying, and it keeps the estate plan intact.

1. INSURE THE TAIL, SELF-FUND THE FRONT

Pay the first year or two yourself (long elimination period) and insure years three through seven, where the catastrophic cost lives. Premiums drop sharply and the real risk is covered.

2. REPURPOSE OLD LIFE INSURANCE

A 1035 exchange moves the cash value of an outdated policy into a hybrid with no tax. The death benefit stays; care coverage is added.

3. PAY WITH PRE-TAX IRA MONEY

Ten annual IRA distributions fund a hybrid policy. Taxes are spread out, required distributions do double duty, and the care benefits come out tax-free.

4. BUSINESS-OWNER DEDUCTION

C-corps can deduct 100% of traditional LTC premiums for owners; S-corps, partnerships and sole proprietors deduct age-based amounts ($5,880 at 61–70 in 2025). The company pays for the plan.

5. SHARED CARE FOR COUPLES

Two policies linked so either spouse can draw on the other's unused benefits. Statistically one spouse needs far more care than the other; shared care prices for the average instead of the worst case.

6. THE PENSION PROTECTION ACT ANNUITY

Since 2010, gains inside an annuity used for qualified long-term care come out **tax-free**. An old annuity with a big gain can be exchanged into an LTC annuity and the gain is never taxed if used for care.

Why they do it at all

Three reasons come up in nearly every planning meeting with families who could write the check:

- **Sequence risk.** Needing $300,000 of care in a year the market is down 25% means selling at the bottom. Insurance pays out regardless of markets.
- **The inheritance stays whole.** A seven-figure care event is the most common way a family's legacy shrinks by a third.

The simple version: the wealthy use leverage, tax code and old policies to turn care into a line item instead of a crisis. Every one of these strategies is available to a family with $500,000 saved, not just $5,000,000.

CHAPTER 8

Real-life examples

Details changed to protect privacy; the numbers and decisions are real.

HYBRID, FUNDED WITH OLD INSURANCE

Robert & Linda, 61 and 59, retired teacher and nurse

Situation: $1.1M in retirement accounts, paid-off home, and a universal life policy Robert bought in 1998 with $142,000 of cash value he "forgot about."

Plan: 1035 exchange of the old policy into a joint hybrid. No new money out of pocket, no tax on the exchange.

Result: $7,100/month of care benefit for each of them for up to six years ($511,000 each), 3% inflation rider, and a $236,000 death benefit if neither needs care. Premium guaranteed never to rise.

TRADITIONAL, BUSINESS-OWNER DEDUCTION

Marcus, 54, owns an HVAC company (S-corp)

Situation: Healthy, high income, watched his mother spend $410,000 on memory care. Wanted maximum coverage per dollar and didn't care about a death benefit.

Plan: Traditional policy, $9,000/month benefit, 5-year pool, 3% compound inflation, 90-day elimination. Company pays the premium; $1,790 is deductible at his age bracket.

Result: $540,000 pool today that grows to about $1.1M by age 80, for $4,260 a year. He plans to add his wife on a shared-care rider next year.

ANNUITY WITH LTC RIDER, HEALTH ISSUE

Dolores, 67, widow, type 2 diabetes

Situation: Declined for traditional and hybrid coverage. $220,000 in a CD paying 2.1% that she called "my nursing home money."

Plan: Moved $150,000 into a fixed annuity with a long-term care doubler. Simplified underwriting, no medical exam.

Result: The $150,000 becomes about $300,000 of care benefit paid over six years if she needs it; otherwise it grows at a guaranteed rate and passes to her daughter. The other $70,000 stayed liquid.

The simple version: there was a workable plan for every one of these families, including the one who had been declined. The common thread is that each of them did it before they needed it.

CHAPTER 9

The most common and costly mistakes

We see the same eight errors over and over. Each one is avoidable.

The mistake	Why it hurts	Do this instead
Waiting until "later"	Every year of delay raises price 6–10% and lowers the odds of qualifying. Half of all declines are for conditions that were fine five years earlier.	Price it in your early 50s even if you don't buy yet
Assuming Medicare covers it	Families discover the truth on day 101 of a nursing stay, when the bill arrives.	Read Chapter 3 twice
Skipping inflation protection	A $6,000 benefit bought at 55 covers a $17,000 bill at 85. The policy "works" and still leaves you short.	3% compound minimum; 5% if you're under 55
Buying only for the nursing home	Most care happens at home. Some older policies pay less, or nothing, at home.	Make sure home care pays at 100% of the benefit
Insuring one spouse	The uninsured spouse is usually the one who needs care longer.	Cover both, or use shared care
Giving assets away too late	Medicaid's 5-year look-back turns a gift into a penalty period with no coverage.	Any gifting strategy needs an elder-law attorney and a 5-year runway
Letting an old policy lapse	Policies bought in the '90s and 2000s often have benefits that can't be purchased today.	Have it reviewed before you drop or replace it
No one knows the plan exists	Benefits go unclaimed because the kids never knew about the policy.	Put the policy, the agent's number and the claims process in one folder the family can find

Expert insight: The single most expensive mistake is not a bad policy; it is no policy, discovered at age 74 with a diagnosis on file. At that point, the only options left are self-funding and Medicaid.

The simple version: plan early, cover both spouses, insist on inflation protection and home care, and tell your family where the paperwork is.

CHAPTER 10

Questions to ask before you buy

Bring this page to any meeting, with us or anyone else. A good advisor will welcome every question.

About the policy

- Is the premium guaranteed, or can it increase?
- What exactly triggers benefits? (Look for "2 of 6 ADLs or cognitive impairment.")
- Does home care pay the same monthly amount as facility care?
- Is the benefit paid as reimbursement (receipts) or cash indemnity (no receipts)?
- What is the elimination period, and is it calendar days or days of paid service?
- What inflation option is included, and what does it cost?
- Is there a return-of-premium or death benefit if I never use it?
- Does it cover care from family members or only licensed providers?

About the company and the advisor

- What is the insurer's A.M. Best rating? (A or better.)
- How long has this company sold long-term care coverage, and has it ever raised rates on existing policyholders?
- Does the policy qualify as "tax-qualified" under IRC §7702B?
- Is this a state Partnership-qualified policy?
- Will you show me at least two carriers side by side?
- How are you paid, and does that change by carrier?
- Who helps my family file a claim in 20 years?

What a good plan looks like on one page

BENEFIT

Monthly amount ≈ cost of a private room in your area, or 70% of it if you'll self-fund the rest

DURATION

3 years minimum; 5–6 years or lifetime continuation if family history includes dementia

GROWTH

3% compound inflation, or a benefit pool large enough that growth isn't needed

The simple version: guaranteed premium, clear triggers, full home-care benefit, inflation protection, an A-rated company, and an advisor who shows you more than one option.

CHAPTER 11

Your next step

You don't need to decide anything today. You need to know your numbers.



A 20-minute Long-Term Care Game Plan call

In one short call we'll put three things on paper for you:

YOUR EXPOSURE

What care costs where you live, projected to the age you'd likely need it.

YOUR OPTIONS

Which of the five ways to pay fit your health, assets and goals, with real quotes from A-rated carriers.

YOUR EXISTING ASSETS

Whether an old life policy, annuity or IRA can fund the plan without new money out of pocket.

Book your Long-Term Care Game Plan call

No cost, no obligation, and no pressure. If the honest answer is "you can self-fund this," we'll tell you.

Reply to the text or email that brought you here, or call (619) 374-8100

Pacific Ridgeway Insurance Solutions · San Diego, CA · Licensed in multiple states

> *"The best time to plan for long-term care was ten years ago. The second-best time is before your next birthday."*

IMPORTANT DISCLOSURES

The fine print, in plain English

This is education, not advice. This guide is for general informational purposes and does not constitute insurance, legal, tax or investment advice. Your situation is unique; speak with a licensed professional before making decisions.

Costs and statistics. Cost-of-care figures are national medians from the Genworth/CareScout Cost of Care Survey 2024 and are rounded. Probability-of-need and duration figures are from the U.S. Department of Health and Human Services Administration for Community Living. Premium examples are from the American Association for Long-Term Care Insurance 2024 Price Index. Actual costs and premiums vary by location, age, health, carrier and policy design.

Examples. The case studies in Chapter 8 are composites based on actual planning engagements. Names and identifying details have been changed. Benefit amounts are illustrative and depend on carrier, underwriting class, age and product design at the time of purchase. They are not a guarantee of any outcome.

Insurance products. Long-term care insurance, hybrid life/long-term care policies and annuities with long-term care riders are issued by insurance companies and are subject to underwriting, exclusions, limitations and the claims-paying ability of the issuing company. Guarantees are backed solely by the issuing insurer. Policy terms, including benefit triggers, elimination periods and inflation options, vary by carrier and state.

Taxes. Tax treatment described here (including IRC §7702B qualified long-term care benefits, §1035 exchanges, Pension Protection Act annuity provisions and premium deductibility) is based on federal law as of 2025 and may change. State treatment differs. Consult a qualified tax professional.

Medicaid and Medicare. Program rules, asset limits and look-back periods are set by federal and state law and change over time. Nothing here should be used to plan Medicaid eligibility without an elder-law attorney licensed in your state.

About us. Pacific Ridgeway Insurance Solutions is an independent insurance agency. Gregory Stevenson is a licensed insurance agent. We are compensated by insurance carriers when a policy is issued; we are not compensated for this guide or for a planning call. We represent multiple carriers and will show you more than one option.

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The Complete Long-Term Care Guide, first edition.